



Reporting of Waiting Lists for Mental Health Services

**As Required by
2026-2027 General Appropriations Act
Senate Bill 1, 89th Legislature, Regular
Session, 2025 (Article II, Health and
Human Services Commission, Rider 47)**

**Texas Health and Human Services
November 2025**



TEXAS
Health and Human
Services

Table of Contents

1. Introduction	2
Data Context and Limitations.....	2
2. Adult and Children’s Community Mental Health Services Waiting List Data	4
Adult Community Mental Health Services.....	4
Children’s Community Mental Health Services	7
Funds Expended to Remove People from Waiting Lists	9
Factors Affecting Community Mental Health Waiting Lists for Adults and Children	9
3. Forensic Inpatient Waiting List Data	11
Factors Impacting Forensic Inpatient Waiting Lists	14
Funds Expended to Remove People from Forensic Inpatient Bed Waiting Lists...	19
List of Acronyms	22

1. Introduction

The *Reporting of Waiting Lists for Mental Health Services* meets the requirements of the 2026-27 General Appropriations Act, Senate Bill (S.B.) 1, 89th Legislature, Regular Session, 2025 (Article II, Health and Human Services Commission, Rider 47). Rider 47 requires Health and Human Services Commission (HHSC) to report on waiting lists for mental health services to the Legislative Budget Board and the Office of the Governor by November 1 of each fiscal year.

Rider 47 requires the report to include waiting list and expenditure data for community mental health services for adults and children, forensic state hospital beds, and maximum-security forensic state hospital beds for each local mental health authority (LMHA), local behavioral health authority (LBHA), state facility, and contracted entity. Required data include:

- Number of people waiting for all services;
- Number of underserved¹ people waiting for additional services;
- Number of people removed from the waiting list;
- Funds expended to remove people on the waiting list in each fiscal quarter included in the reporting period;
- Average number of days spent on a waiting list; and
- Explanation whether waiting lists are due to operational or other short-term factors and long-term waiting lists due to insufficient capacity.

The reporting period for this report is fiscal year 2025.

Data Context and Limitations

The LMHA and LBHA community mental health waiting lists are point-in-time² calculations of the total number of people waiting for mental health services at the end of the respective quarter of the reporting period. The waiting lists are subject to daily fluctuations as people are added or removed for various reasons (e.g.,

¹ Term used to designate persons who qualified for a higher level of care, based on assessment scores, but received a lower level of care due to staff or resource limitations.

² Calculated at the end of the fiscal quarter.

admitted to services, no longer interested in services, receiving services elsewhere, no longer able to be contacted).

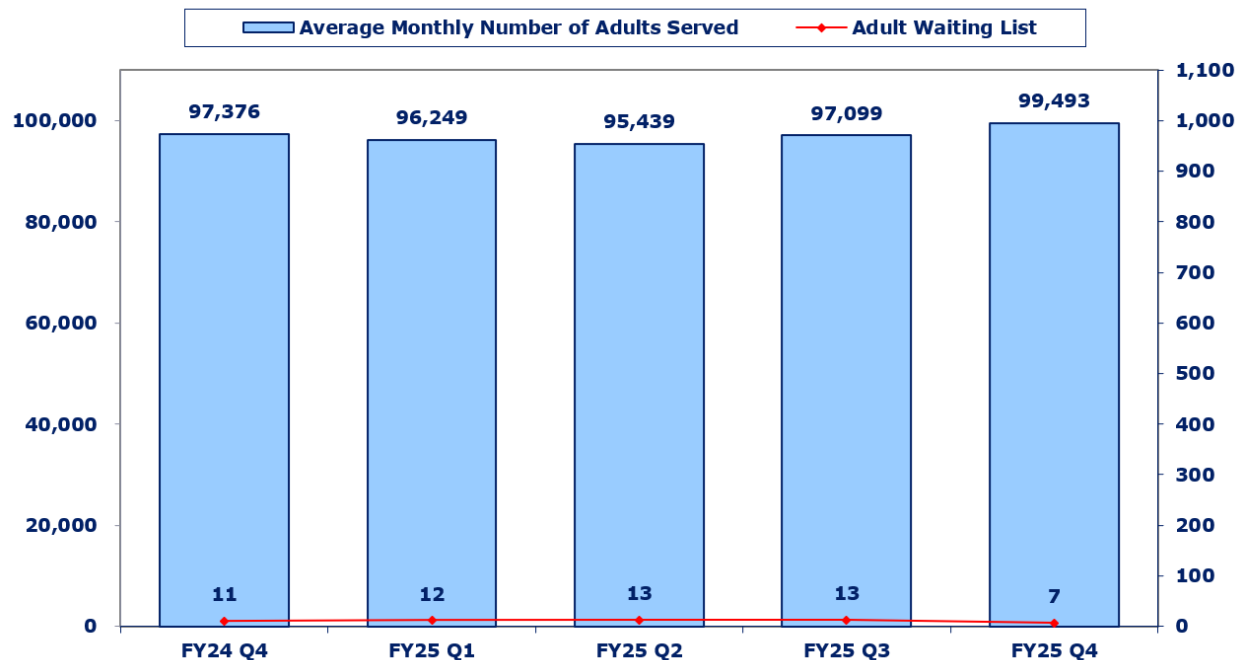
If the demand for services increases beyond the total funded capacity and available staff resources, a waiting list may remain or increase despite the effective use of funding and an LMHA's or LBHA's success in meeting its contracted targets.

2. Adult and Children's Community Mental Health Services Waiting List Data

Adult Community Mental Health Services

Figure 1 shows the average monthly number of adults receiving community mental health services and the number of adults on the waiting list at the end of the fiscal quarter.³

Figure 1. Adult Community Mental Health Number Served and Waiting List for All Services FY25 (Q1-Q4) ⁴



³ Source: Client Assignment and Registration, September 2025.

⁴ "Average Monthly Number of Adults Served" uses Legislative Budget Board performance measure D.2.1. Strategy: Community Mental Health Services – Average Monthly Number of Adults Receiving Community Mental Health Services.

Table 1. Adult Community Mental Health Number Served and Waiting List for All Services for FY25 (Q1-Q4)

Category	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Average Monthly Number of Adults Served	96,249	95,439	97,099	99,493
Adult Waiting List for All Services	12	13	13	7

Tables 2.1, 2.2, and 3 show the required report elements for the community mental health services for adults in LMHAs and LBHAs with waiting lists.

Table 2.1 Number of Adults Waiting for All Services or Removed from a Waiting List and Average Number of Days on a Waiting List by LMHA or LBHA for FY25 (Q1-Q2)⁵

LMHA or LBHA	No. Waiting for All Services FY25 Q1	No. Waiting for All Services FY25 Q2	No. Removed from Waiting List FY25 Q1	No. Removed from Waiting List FY25 Q2	Average No. of Days on Waiting List FY25 Q1	Average No. of Days on Waiting List FY25 Q2
Gulf Bend Center	12	13	1	3	100	120
Total	12	13	1	3	100	120

Table 3.2 Number of Adults Waiting for All Services or Removed from a Waiting List and Average Number of Days on a Waiting List by LMHA or LBHA for FY25 (Q3-Q4)⁶

LMHA or LBHA	No. Waiting for All Services FY25 Q3	No. Waiting for All Services FY25 Q4	No. Removed from Waiting List FY25 Q3	No. Removed from Waiting List FY25 Q4	Average No. of Days on Waiting List FY25 Q3	Average No. of Days on Waiting List FY25 Q4
Gulf Bend Center	13	7	7	6	100	49
Total	13	7	7	6	100	49

⁵ LMHAs and LBHAs with no waiting lists for all services from FY 25 Q1 to FY 25 Q2 are excluded.

⁶ LMHAs and LBHAs with no waiting lists for all services from FY 25 Q3 to FY 25 Q4 are excluded.

Table 4. Number of Underserved⁷ Adults Waiting for Additional Services by LMHA or LBHA^{8,9}

LMHA or LBHA	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Anderson Cherokee Community Enrichment Services (ACCESS)	1	2	1	1
Andrews Center Behavioral Healthcare System	53	38	19	15
Austin Travis County Integral Care	286	265	227	199
Betty Hardwick Center	1	4	9	4
Border Region Behavioral Health Center	26	23	14	15
Camino Real Community Services	22	16	12	13
Center for Health Care Services	38	36	56	80
Center for Life Resources	11	8	6	2
Central Counties Services	0	0	0	1
Central Plains Center	1	1	1	2
Denton County MHMR Center	32	40	45	35
Emergence Health Network	29	27	23	33
Gulf Bend Center	12	8	2	3
Gulf Coast Center	2	1	0	1
Heart of Texas Behavioral Health Network	0	0	6	5
Helen Farabee Centers	0	1	1	1
MHMR Authority of Brazos Valley	51	46	39	40
Nueces Center for Mental Health and Intellectual Disabilities	93	61	43	38
Pecan Valley Centers for Behavioral and Developmental Healthcare	132	98	62	71
PermiaCare	1	1	0	0
Community Healthcore	41	30	8	1
Spindletop Center	86	47	22	22
StarCare Specialty Health System	333	284	250	152
Texas Panhandle Centers	3	1	0	1
Texoma Community Center	2	3	1	1
Tri-County Behavioral Healthcare	55	53	84	94
Tropical Texas Behavioral Health	216	196	188	171

⁷ Term used to designate persons who qualified for a higher level of care, based on assessment scores, but received a lower level of care due to staff or resource limitations.

⁸ Data does not include adults in a lower level of care due to "consumer refused."

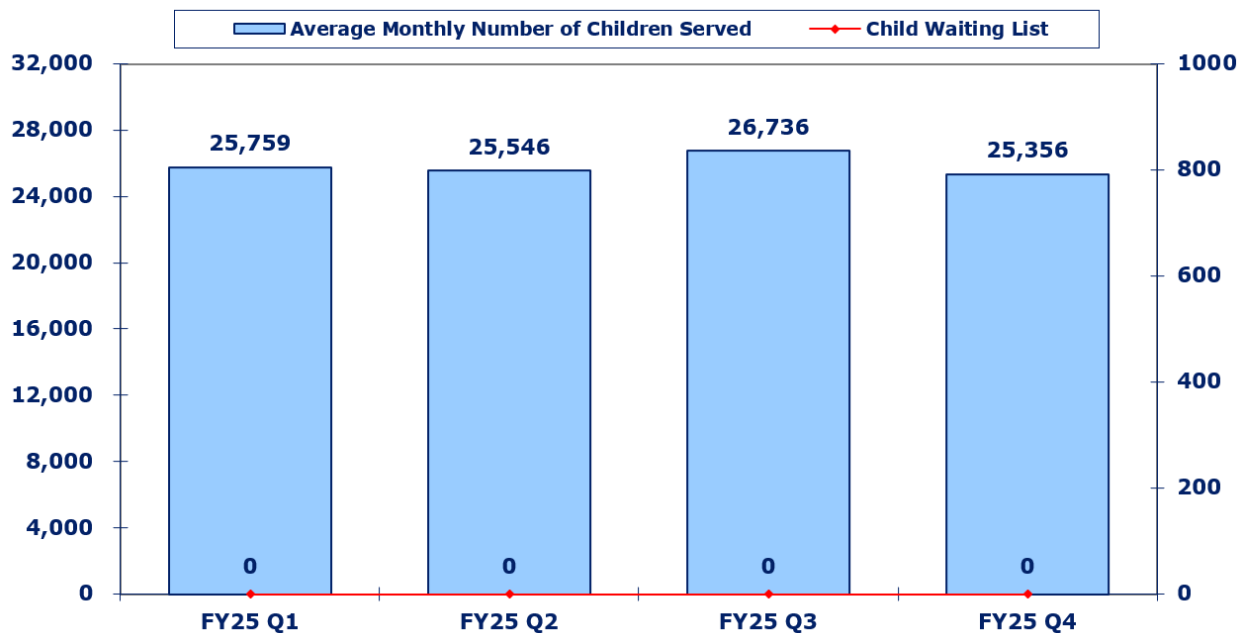
⁹ LMHAs and LBHAs with no underserved waiting lists from FY25 Q1 to Q4 are excluded.

LMHA or LBHA	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
West Texas Centers	1	1	1	8
Total	1,528	1,291	1,120	1,009

Children's Community Mental Health Services

The following figure shows the average monthly number of children receiving community mental health services and the number of children waiting for community mental health services at the end of the fiscal quarter.¹⁰

Figure 2. Children's Community Mental Health Number Served and Waiting List for All Services for FY25 (Q1-Q4) ¹¹



¹⁰ Source: Client Assignment and Registration, March 2025.

¹¹ "Average Monthly Number of Children Served" uses Legislative Budget Board performance measure D.2.1 Strategy: Community Mental Health Services – Average Monthly Number of Children Receiving Community Mental Health Services.

Table 5. Children’s Community Mental Health Number Served and Waiting List for All Services for FY25 (Q1-Q4)

Category	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Average Monthly Number of Children Served	25,759	25,546	26,736	25,356
Child Waiting List	0	0	0	0

Tables 5.1, 5.2, and 6 summarize the required report elements for the community mental health services waiting list for children.

Table 6.1. Number of Children Waiting for All Services or Removed from a Waiting List and Average Number of Days on a Waiting List by LMHA or LBHA for FY25 (Q1-Q2)

LMHA or LBHA	No. Waiting for All Services FY25 Q1	No. Waiting for All Services FY25 Q2	No. Removed from Waiting List FY25 Q1	No. Removed from Waiting List FY25 Q2	Average No. of Days on Waiting List FY25 Q1	Average No. of Days on Waiting List FY25 Q2
Total	0	0	0	0	0	0

Table 7.2. Number of Children Waiting for All Services or Removed from a Waiting List and Average Number of Days on a Waiting List by LMHA or LBHA for FY25 (Q3-Q4)

LMHA or LBHA	No. Waiting for All Services FY25 Q3	No. Waiting for All Services FY25 Q4	No. Removed from Waiting List FY25 Q3	No. Removed from Waiting List FY25 Q4	Average No. of Days on Waiting List FY25 Q3	Average No. of Days on Waiting List FY25 Q4
Total	0	0	0	0	0	0

Table 8. Number of Underserved¹² Children Waiting for Additional Services by LMHA or LBHA^{13,14}

LMHA or LBHA	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Anderson Cherokee Community Enrichment Services (ACCESS)	0	1	0	0
Andrews Center Behavioral Healthcare System	4	1	0	0
Austin Travis County Integral Care	1	0	1	0
Border Region Behavioral Health Center	0	1	1	0
Central Counties Center for MHMR Services	0	1	0	0
Community Healthcore	107	105	172	124
Denton County MHMR Center	4	0	1	1
Emergence Health Network	73	4	0	0
Gulf Coast Center	0	0	0	1
Heart of Texas Behavioral Health Network	1	43	59	63
Helen Farabee Centers	0	0	2	2
Lakes Regional Community Center	1	0	0	0
Nueces Center for Mental Health and Intellectual Disabilities	1	0	0	0
Pecan Valley Centers for Behavioral & Developmental Healthcare	26	10	9	3
PermiaCare	0	1	0	0
Spindletop Center	203	42	53	47
Starcare Specialty Health System	0	0	1	0
Texas Panhandle Centers	0	1	3	2
Texoma Community Center	3	0	1	3
Tri-County Behavioral Healthcare	0	2	1	0
Tropical Texas Behavioral Health	30	19	22	20
West Texas Centers	1	3	2	5
Total	455	234	328	271

¹² Term used to designate persons who qualified for a higher level of care, based on assessment scores, but received a lower level of care due to staff or resource limitations.

¹³ Data does not include children in a lower level of care due to "consumer refused."

¹⁴ LMHAs and LBHAs with no underserved waiting lists from FY 25 Q1 to Q4 are excluded.

Funds Expended to Remove People from Waiting Lists

The 2026-27 General Appropriations Act, S.B. 1, 89th Legislature, Regular Session, 2025, appropriated \$728,489,068 for Community Mental Health Services (D.2.1) in fiscal year 2026. This includes \$402,353,288 for Adult Community Mental Health Services and \$131,477,768 per fiscal year for Children's Community Mental Health Services. HHSC's Utilization Management Program Manual requires LMHAs and LBHAs to prioritize the reduction of the waiting list as soon as service capacity allows.

In accordance with the federal Coronavirus Response and Relief Supplemental Appropriations Act within the Consolidated Appropriations Act, 2021 and the American Rescue Plan Act, 2021, the Substance Abuse and Mental Health Services Administration awarded Texas a combined total of \$207.8 million in Community Mental Health Services Block Grant supplemental funds. Of this amount, HHSC allocated \$80.7 million for the Mental Health Outpatient Capacity Expansion project. LMHAs and LBHAs continued using these funds in the first three quarters of fiscal year 2025. American Rescue Plan Act funding expired on March 24, 2025, and no expenditures occurred after that date.

Factors Affecting Community Mental Health Waiting Lists for Adults and Children

LMHAs and LBHAs with waiting lists report ongoing workforce shortage challenges, including difficulties in recruiting and retaining behavioral health providers, particularly Licensed Practitioners of the Healing Arts and Qualified Mental Health Professionals. These challenges stem from a competitive job market in the mental healthcare industry and opportunities from other employers statewide. LMHAs and LBHAs had limited funding to retain existing staff or increase targeted recruiting efforts.

LMHAs and LBHAs across the state face professional mental health workforce shortages, with the impact on community mental health waiting lists being greatest for rural LMHAs. Gulf Bend Center, the one LMHA with a current waiting list, is a rural provider in a designated health professional shortage area. Gulf Bend Center surpassed its contracted service targets every month in the first two quarters of

fiscal year 2025. However, the percentage of the adult service target met fell to 92 percent in quarter three before rebounding to 96 percent in quarter four.

To minimize the adult and child waiting lists, HHSC monitored the community mental health waiting list monthly and contacted LMHAs and LBHAs reporting waiting lists to obtain details on the causes and efforts taken to reduce them. HHSC provided technical assistance to these LMHAs and LBHAs to assist them in addressing their workforce needs and expanding their service capacity.

3. Forensic Inpatient Waiting List Data

Tables 7-12¹⁵ below show the number of people waiting, number of people removed, and average number of days spent waiting on the forensic inpatient bed waiting lists for each quarter of fiscal year 2025. HHSC's Health and Specialty Care System (HSCS) maintains two distinct lists of pending admissions for people on inpatient forensic commitments: one for people pending admission to a non-maximum-security unit (non-MSU); and one for people pending admission to a maximum-security unit (MSU).

People may be admitted to a different facility than the facility they are noted to be waiting for. This may be due to the individual specific need for a special population bed, admission to a contract bed with a statewide catchment area, or other factors. Additionally, people waiting for admission to an MSU are not added to a facility-specific waiting list until closer to their admission because all MSUs serve a statewide catchment area, rather than a region-specific catchment area.

The number of people on the waitlist is a snapshot of the last business day of the quarter. The number of people removed from a forensic inpatient bed waiting list includes people removed due to admission, case resolution, restoration of competency in jail, and various other dispositions.

The average number of days spent on a forensic inpatient bed waiting list is measured from the date the court notifies HSCS of the commitment to the date the person is admitted to the hospital. If there is not an average number of days noted in the tables below, that means no individuals were admitted to the facility from the waitlist during that quarter.

¹⁵ The numbers reported in Tables 10-12 reflect the most updated and accurate data available. To the extent numbers reported here for Q1 and Q2 differ from those reported in the May 2025 report, they represent corrections to numbers previously reported.

Table 7. Number of People on Non-MSU Forensic Inpatient Waiting List

Facility/Contractor	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Austin State Hospital	197	214	260	283
Big Spring State Hospital	31	25	19	19
Dallas Behavioral Hospital	1	0	0	3
Dallas Behavioral Hospital (State Hospital Contract Beds)	3	2	3	7
El Paso Psychiatric Center	68	58	58	79
Harris County Psychiatric Center	0	0	0	0
Hickory Trail Hospital	14	4	3	9
Integral Care	5	3	4	2
John S. Dunn Behavioral Center	175	183	210	217
Kerrville State Hospital	2	2	0	5
Montgomery County Hospital	13	16	26	22
Palestine Regional	3	16	9	16
Perimeter Behavioral Hospital of Arlington	0	0	0	3
Perimeter Behavioral Hospital of Dallas	0	0	0	7
Rio Grande State Center	0	0	0	0
Rio Vista Behavioral Health	0	0	0	0
Rusk State Hospital	110	112	113	123
San Antonio State Hospital	209	176	182	173
Terrell State Hospital	240	215	176	213
Wichita Falls State Hospital	176	169	142	104
Total	1,247	1,195	1,205	1,285

Table 8. Number of People Removed from Non-MSU Forensic Inpatient Waiting List

Facility/Contractor	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Austin State Hospital	69	62	67	103
Big Spring State Hospital	43	61	62	46
Dallas Behavioral Hospital	4	0	0	0
Dallas Behavioral Hospital (State Hospital Contract Beds)	1	8	8	5
El Paso Psychiatric Center	19	18	21	26
Harris County Psychiatric Center	11	16	16	9
Hickory Trail Hospital	14	20	26	12
Integral Care	53	41	40	23
John S. Dunn Behavioral Center	108	128	133	141

Facility/Contractor	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Kerrville State Hospital	6	7	7	2
Montgomery County Hospital	64	39	41	51
Palestine Regional	40	32	36	30
Perimeter Behavioral Hospital of Arlington	0	0	0	39
Perimeter Behavioral Hospital of Dallas	7	17	18	14
Rio Grande State Center	0	2	2	5
Rio Vista Behavioral Health	0	1	0	0
Rusk State Hospital	15	8	8	19
San Antonio State Hospital	71	71	78	65
Terrell State Hospital	74	60	65	31
Wichita Falls State Hospital	85	79	86	104
Total	684	670	714	725

Table 9. Average Number of Days on Non-MSU Forensic Inpatient Waiting List

Facility/Contractor	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Austin State Hospital	84	224	75	321
Big Spring State Hospital	124	149	66	54
Dallas Behavioral Hospital	206	0	152	0
Dallas Behavioral Hospital (State Hospital Contract Beds)	0	219	211	204
El Paso Psychiatric Center	317	495	265	405
Harris County Psychiatric Center	105	92	159	176
Hickory Trail Hospital	250	228	193	214
Integral Care	182	155	199	220
John S. Dunn Behavioral Sciences Center	138	137	165	147
Kerrville State Hospital	34	46	39	120
Montgomery County Hospital	307	301	256	251
Palestine Regional	248	248	219	229
Perimeter Behavioral Hospital of Arlington	0	0	0	121
Perimeter Behavioral Hospital of Dallas	258	202	0	174
Rio Grande State Center	0	780	258	589
Rio Vista Behavioral Health	0	0	0	0
Rusk State Hospital	592	341	428	409
San Antonio State Hospital	236	334	320	285
Terrell State Hospital	211	194	200	134

Facility/Contractor	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Wichita Falls State Hospital	150	181	173	110
Total	198	198	190	178

Table 10. Number of People on MSU Forensic Inpatient Waiting List¹⁶

Facility	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Kerrville State Hospital	12	8	1	3
Rusk State Hospital	9	11	25	22
Vernon State Hospital	38	56	45	39
Wichita Falls State Hospital	2	7	3	1
Maximum Security ¹⁷	457	423	408	419
Total	518	505	482	484

Table 11. Number of People Removed from MSU Forensic Inpatient Waiting List

Facility	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Kerrville State Hospital	41	29	20	5
Rusk State Hospital	49	44	45	38
Vernon State Hospital	84	97	125	105
Wichita Falls State Hospital	20	18	16	20
Maximum Security ¹⁷	62	46	86	62
Total	256	234	292	230

Table 12. Average Number of Days from notification to admission on MSU Forensic Inpatient Waiting List

Facility	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Kerrville State Hospital	253	173	261	148
Rusk State Hospital	288	222	223	195
Vernon State Hospital	338	288	272	221
Wichita Falls State Hospital	288	239	183	138

¹⁶ HHSC included San Antonio State Hospital, Rio Grande State Center, Terrell State Hospital, and Perimeter Hospital in Table 10 in the May 2025 report, and has updated the November 2025 report to reflect the appropriate facilities.

¹⁷ Individuals waiting for admission to an MSU are not added to a facility-specific waiting list until closer to their admission because all MSUs serve a statewide catchment area, rather than a region-specific catchment area.

Facility	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Maximum Security ^{17,18}	0	0	0	0
Total	302	247	253	202

Factors Impacting Forensic Inpatient Waiting Lists

Below are the long-term factors that impact the forensic waiting list. Currently, there are no short-term factors impacting the forensic waiting list.

Deferred Maintenance

State hospitals are required to comply with the federal Life Safety Code and regulatory requirements, such as ensuring the mitigation of ligature risks.¹⁹ State hospital beds are taken offline when mitigation and remediation efforts or maintenance are required.

The Supplemental Appropriations Act, House Bill 500, 89th Legislature, Regular Session, 2025, appropriated \$98,000,000 for deferred maintenance and to Strategy G.4.2, Facility Repairs and Renovation, to be used for state hospitals and state supported living centers. Currently, the cost of anti-ligature remediation and other necessary repairs for the state hospitals exceeds what has been appropriated, and the state hospitals experience challenges maintaining buildings with aging infrastructure and building systems, which further impacts the number of people who can be served. HSCS has a deferred maintenance need of approximately \$800,000,000 and requests appropriations each legislative session to maintain a consistent cycle of projects to meet this need.

County Planning, Coordination, and Education

Planning, coordination, and education among county stakeholders can impact the demand for inpatient competency restoration services. Collaboration with stakeholders can help to address effective and timely services depending on the

¹⁸ Table 12 covers admissions to specific hospitals. Maximum Security is not a physical location. Because of this, people do not directly admit from the maximum security list.

¹⁹ The Centers for Medicare and Medicaid Services notes that a ligature risk (point) may include anything which could be used to create a sustainable attachment point such as a cord, rope, or other material for the purpose of hanging or strangulation.

individual's needs and placements, such as when an individual's competency to stand trial is restored prior to admission to a state hospital, or when a person is not likely to restore to competency in the foreseeable future but may be able to successfully live and receive treatment in the community. Education on the competency restoration process and its appropriate use can also reduce the use of inpatient competency restoration and increase the use of outpatient and jail-based competency restoration programs and alternative dispositions. The HSCS Office of Forensic Services and Coordination (OFSC) proactively reaches out to stakeholders to provide education and enhance collaboration with counties.

Additionally, OFSC engages counties through the statewide Jail In-Reach Program, which aims to educate and collaborate with external stakeholders and community-based teams to support active forensic waitlist monitoring and services through:

- Clinical consultation services that may assist with psychiatric stabilization;
- Trial competency re-evaluations in the event of immediate restoration while awaiting state hospital transfer;
- Legal education on options for alternative case dispositions; and
- Enhanced follow-ups on people restored to competency and returned to jail to prevent clinical decompensation and unnecessary rehospitalization.

The OFSC and Behavioral Health Services Forensic and Jail Diversion Services promote statewide initiatives that support local communities in behavioral health and justice planning to reduce and prevent justice-involvement for people with behavioral health needs, with the downstream impact of reducing the number of people in the competency restoration process. OFSC and Forensic and Jail Diversion Services have conducted local strategic planning with counties through Sequential Intercept Model Mapping Workshops.

Sequential Intercept Model Mapping Workshops support counties in developing strategic plans that increase the use of diversion and reduce reliance on inpatient competency restoration services. HHSC plans to continue and expand community-based Sequential Intercept Model mapping exercises in fiscal year 2026 to support local jurisdictions in reducing their need for inpatient competency restoration.

OFSC, in collaboration with the Texas Judicial Commission on Mental Health, launched the Eliminate the Wait Campaign in 2021, which provides municipal and county behavioral health and justice stakeholders with training and technical

assistance on strategies that reduce reliance on inpatient competency restoration services. A [toolkit](#) is publicly available and training is provided on request.

The Texas Behavioral Health and Justice Technical Assistance Center provides statewide training and technical assistance on competency restoration processes, diversion, and other issues that impact forensics and justice.

Current Construction

HSCS is ensuring current and future state hospital capacity through the following projects:

San Antonio State Hospital

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$15,000,000 to Strategy G.4.2, Facility Repairs and Renovation, to convert the 40-bed Alamo Hall at San Antonio State Hospital to an MSU, and HHSC is designating this unit to serve the geriatric MSU population. The architecture and engineering (A/E) contract was executed in August 2023, and the construction contract was awarded in summer 2025. Construction completion is estimated for fall 2026.

Dallas State Hospital

The Dallas State Hospital will be a new 292-bed inpatient psychiatric hospital with 200 adult beds, at least 75 of which will serve the non-MSU forensic population, and 92 pediatric beds. Both units are under construction with completion estimated for November 2025 for the adult unit, and April 2026 for the pediatric unit.

Wichita Falls State Hospital

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$452,000,000 to Strategy G.4.2 to construct a 225-bed replacement hospital for the Wichita Falls State Hospital to include 25 MSU beds, 16 civil beds, 25 adolescent beds, and 159 non-MSU forensic beds. The A/E contract was executed in December 2023, and the construction manager at risk (CMR) contract was executed in May 2024. The groundbreaking ceremony took place on September 24, 2024. Construction completion is estimated for fall 2027.

Terrell State Hospital

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$573,000,000 to Strategy G.4.2 to construct a 275-bed replacement hospital for the Terrell State

Hospital campus to include 50 MSU beds, 25 civil beds, 50 adolescent beds, and 150 non-MSU forensic beds. The A/E contract was executed in January 2024, and the CMR contract was executed in July 2024. The groundbreaking ceremony took place on August 28, 2024. Construction completion is estimated for summer 2027.

Rio Grande State Center

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$120,000,000 to Strategy G.4.2 to expand the Rio Grande State Center by 50 MSU beds. The A/E contract was executed in February 2024, and the CMR contract was executed in August 2024. Construction began summer 2025 and completion is estimated for summer 2027.

Panhandle State Hospital

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$159,000,000 to Strategy G.4.2 to construct a new 75-bed hospital in Amarillo to include 50 non-MSU forensic beds. The A/E contract was executed in December 2023, and the CMR contract was executed in July 2024. The land lease agreement was executed in January 2025. The groundbreaking ceremony took place on June 10, 2025. Construction completion is estimated for summer 2027.

Lubbock Psychiatric Center

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$121,000,000 to Strategy G.2.4 to construct a new 50-bed MSU hospital in Lubbock. The A/E contract was executed in February 2024, and the CMR contract was executed in July 2024. The land lease agreement between TDCJ and HHSC was executed in May 2025. Construction began summer 2025 and completion is estimated for fall 2027.

Additional Capacity

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$239,000,000 in grants to Strategy G.4.2 for construction of up to 332 inpatient beds, half of which must serve forensic admissions, in the Rio Grande Valley, Victoria County, Montgomery County, and City of Beaumont. All grant award contracts have been executed.

Additionally, S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$45,000,000 for Strategy G.4.2 to construct 30 additional beds at the Sunrise

Canyon Facility in Lubbock, Texas. At least 50 percent, 15 beds, will have forensic capacity. Construction completion is estimated for spring 2026.

The General Appropriations Act, S.B. 1, 89th Legislature, Regular Session, 2025 increased funding to Strategy G.2.2., Mental Health Community Hospitals, to maintain existing contract beds and to contract for additional inpatient beds, including those specifically providing competency restoration services, currently under construction in the Rio Grande Valley and Montgomery County as they are expected to reach construction completion during the 2026-27 biennium. As of the end of fiscal year 2025, HSCS contracts for 386 competency restoration beds.

Additionally, S.B. 1, 89th Legislature, Regular Session, 2025 appropriated \$5,000,000 for Strategy D.2.1, Community Mental Health Services in FY26 to fund additional crisis stabilization facilities and services and provide additional inpatient competency restoration beds in Tarrant County. Upon contract execution, Tarrant County will contract for five inpatient competency restoration beds per day.

Unscheduled Admissions

A growing number of unscheduled admissions or “walk-ins” impact admissions to state hospitals. Walk-ins are typically referrals primarily from law enforcement, family members, or the individual themselves appearing at a state hospital for the purpose of admission and may be either admitted or denied (Table 13). State hospitals must comply with the [Emergency Medical Treatment and Active Labor Act](#). This means if a person needs immediate crisis stabilization, state hospitals must assess the individual and provide stabilizing care, if possible, or transfer that person to another facility for treatment with that facility's agreement.

Table 13. Total Walk-ins (Admitted and Denied)

Facility	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Austin State Hospital	39	32	31	32
Big Spring State Hospital	1	4	5	3
El Paso Psychiatric Center	115	121	119	136
Kerrville State Hospital	0	0	0	0
Wichita Falls State Hospital	0	0	0	1
Rio Grande State Center	0	0	0	0
Rusk State Hospital	5	8	11	1
San Antonio State Hospital	34	54	44	46
Terrell State Hospital	378	357	422	368

Facility	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Total	572	576	632	587

Beds occupied by walk-in admissions limit the number of beds available for planned admissions from the civil waitlist, which are typically coordinated through the LMHA or LBHA, and forensic waitlists, which are typically coordinated with the designated party in the county of an individual's commitment (Table 14).

Table 14. Walk-in Admissions

Facility	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4
Austin State Hospital	13	17	8	15
Big Spring State Hospital	0	0	4	1
El Paso Psychiatric Center	56	55	57	73
Kerrville State Hospital	0	0	0	0
Wichita Falls State Hospital	0	0	0	1
Rio Grande State Center	0	0	0	0
Rusk State Hospital	4	5	10	1
San Antonio State Hospital	21	33	32	33
Terrell State Hospital	300	251	293	286
Total	394	361	404	410

Funds Expended to Remove People from Forensic Inpatient Bed Waiting Lists

Table 15, below, details the funds expended to remove people from the forensic waiting lists in each quarter of fiscal year 2025. Please note that the expenditure data represents spending on a cash basis by fiscal quarter for any open appropriation year.

State hospital operations includes all expenditures from Strategy G.2.1, Mental Health State Hospitals, including funds used for salaries, supplies, and other items. It excludes funds used for contract beds. State hospital operations expenditures include funds used to serve youth and civilly committed and voluntarily admitted adults at state hospitals. These are in addition to forensic admissions.

State hospital contract beds includes expenditures from Strategy G.2.2, Mental Health Community Hospitals, that fund the contracts under HSCS for the operation of beds at the John S. Dunn Behavioral Sciences Center, Montgomery County

Mental Health Treatment Facility, Palestine Regional Medical Center, University of Texas Health Science Center at Tyler, Perimeter Behavioral Hospital, and additional beds subcontracted at facilities through HHSC contracts with North Texas Behavioral Health Authority, Integral Care, and My Health My Resources Tarrant County. State hospital contract beds expenditures include funds used to serve civil admissions at John S. Dunn Behavioral Sciences Center and University of Texas Health Science Center at Tyler, in addition to forensic admissions.

State hospital deferred maintenance and state hospital emergency repairs include expenditures from Strategy G.4.2. Additional deferred maintenance and emergency repair expenditures may be included within the state hospital operations category if funded within a state hospital's existing budget.

Additional efforts to remove people from the forensic inpatient bed waiting list include Inpatient Competency Restoration (ICR), Outpatient Competency Restoration (OCR) and Jail-based Competency Restoration (JBCR). Table 16, below, details the funds expended to remove people from the forensic waiting list through ICR, which includes expenditures from Strategy G.2.2, Mental Health Community Hospitals, OCR, which includes expenditures from Strategy D.2.1. Community Mental Health Services Adults and Strategy D.2.3. Community Mental Health Crisis Services, as well as JBCR, which includes expenditures from Strategy D.2.3. Community Mental Health Crisis Services.

ICR, OCR, and JBCR competency restoration services include legal education and clinically appropriate mental health and substance use treatment services for people determined to be incompetent to stand. ICR programs provide competency restoration services in a hospital setting. OCR programs provide competency restoration services in a community or residential setting. JBCR programs provide competency restoration services in a designated space in a jail separate from general population.

Table 15. State Hospital Forensic Waiting Lists Related Expenditures

Expenditures	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4	TOTAL FYTD 25
State Hospital Operations	\$151,092,132	\$164,051,442	\$160,562,574	\$176,638,709	\$652,344,857
State Hospital Contract Beds	\$15,800,685	\$21,836,529	\$33,000,001	\$28,165,248	\$98,802,463
State Hospital Construction	\$56,120,382	\$150,251,227	\$28,285,250	\$33,289,428	\$267,946,287
State Hospital Deferred Maintenance	\$8,510,842	\$10,398,245	\$6,551,603	\$4,390,468	\$29,851,158
State Hospital Emergency Repairs	\$3,280	\$424,642	\$1,273,880	\$535,055	\$2,236,857
TOTAL	\$231,527,321	\$346,962,085	\$229,673,308	\$243,018,908	\$1,051,181,622

Table 16. JBCR, OCR, and ICR Forensic Waiting Lists Related Expenditures²⁰

Expenditures	FY25 Q1	FY25 Q2	FY25 Q3	FY25 Q4	TOTAL FYTD 25
JBCR	\$890,764	\$917,517	\$1,053,770	\$814,854	\$3,676,906
OCR	\$1,625,332	\$1,118,084	\$745,389	\$745,389	\$4,234,195
ICR	\$3,337,560	\$3,337,560	\$2,225,040	\$2,225,040	\$11,125,200
TOTAL	\$5,853,656	\$5,373,161	\$4,024,199	\$3,785,283	\$19,036,301

²⁰ Expenditures are as of 10/20/2025 and are subject to change until fiscal year closeout is finalized

List of Acronyms

Acronym	Full Name
A/E	Architecture and Engineering
CMR	Construction Manager At-Risk
FY	Fiscal Year
FYTD	Fiscal Year to Date
HHSC	Health and Human Services Commission
HSCS	Health and Specialty Care System
ICR	Inpatient Competency Restoration
JBCR	Jail-based Competency Restoration
LBHA	Local Behavioral Health Authority
LMHA	Local Mental Health Authority
MSU	Maximum-Security Unit
Non-MSU	Non-Maximum-Security Unit
OCR	Outpatient Competency Restoration
OFSC	Office of Forensic Services and Coordination
Q	Quarter
S.B.	Senate Bill